

# The Silent Language of Healing - Mastering Non-Verbal Communication in Medicine

By Dr. Aram Younis Mohammad, Dr. Fateh Ramzi Fateh

Department of Medical Education – College of Medicine - HMU

# List of contents

- Lecture objectives
- Introduction
- Components of non-verbal communication
- Case-scenarios

# Learning Objectives: By the end of this lecture, students will be able to:

- 1. Define non-verbal communication (NVC) and list its core components.
- 2. Explain the critical impact of NVC on the patient-physician relationship, diagnostic accuracy, and patient outcomes.
- 3. Identify their own non-verbal cues and interpret those of patients.
- 4. Apply strategies to align their non-verbal behavior with their verbal messages to build trust and rapport.

# Introduction: The Power of the Unspoken

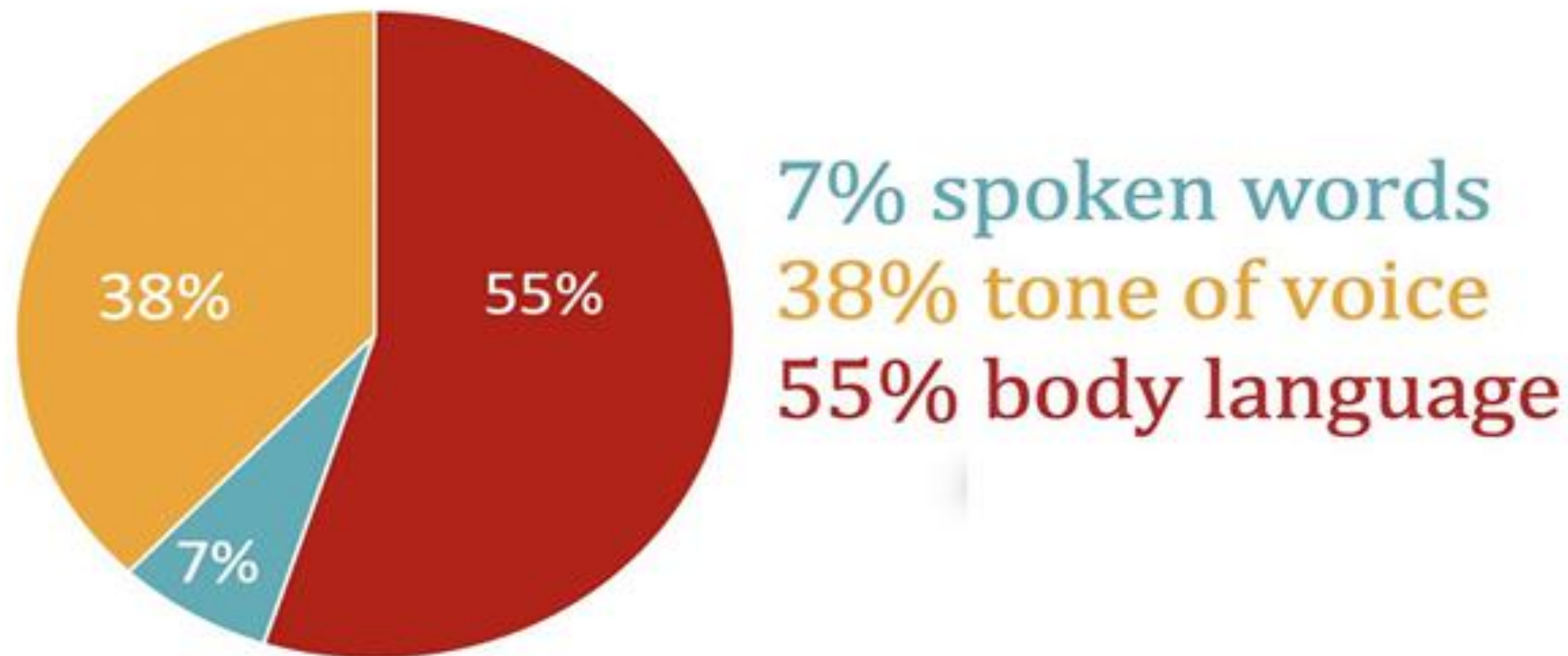
- Hook: "Before you even say 'Hello,' your patient has already begun to form an impression. Studies show that between 60-93% of all communication is non-verbal. What does your silent language say about you?"
- The Two-Channel Theory: We communicate on two channels simultaneously:
  - 1. Verbal Channel: The content of our words.
  - 2. Non-Verbal Channel: The context for those words. When the two conflict, the non-verbal channel almost always wins.

# Introduction: The Power of the Unspoken

- **Why is this crucial in medicine?**
- Builds **trust** and rapport: Patients sense whether you are attentive, empathetic, open or rushed.
- Helps in **assessment**: Observing patient nonverbal signals (pain grimace, distress, discomfort) adds to history-taking.
- Influences **diagnosis & management**: Nonverbal behavior of clinician can affect patient disclosure, adherence.
- Avoids **miscommunication**: Even when you speak clearly, nonverbal cues might send contradictory signals.

# COMMUNICATION

Dr Albert Mehrabian's 7-38-55% Communication Rule  
**Elements of Personal Communication**



# The Core Components of Non-Verbal Communication

- Kinesics / Body language (facial expression, gestures, posture)
- Eye contact / Gaze
- Touch (haptics)
- Proxemics (personal space)
- Chronemics (time use) (e.g., punctuality, pause)
- Paralanguage (tone, pitch, speed of speech)
- Appearance / Dress
- Written/non-verbal aspects of communication (charts, diagrams, colour)
- Silence

# 1. Kinesics

- Facial expressions: universal signals of happiness, anger, fear, sadness.

Gestures: waving, nodding, shaking head, hand movements.

- Posture: how you stand/sit—open vs closed posture; leaning in vs away.

- In a clinical setting:

- Open posture toward patient → conveys attentiveness.
- Crossing arms or turning away may signal disengagement.
- Matching (mirroring) subtle patient posture can help rapport.



# 1. Kinesics

- Reading the Patient: Lack of eye contact may indicate discomfort, deception, or cultural norm; prolonged staring can be a sign of aggression or fear.
- Gestures:
- Clinical Application: Use open palm gestures to appear welcoming. Nodding shows understanding. Avoid crossing arms (defensive), pointing (accusatory), or fidgeting (nervousness, boredom).
- Reading the Patient: Hand-wringing (anxiety), clenched fists (anger, pain), tremors (neurological issues, anxiety).

## 2. Eye Contact, Gaze & Facial Orientation

- Eye contact expresses interest, respect, attention.
- Too much or too little may cause discomfort—cultural differences matter.
- Clinician should avoid looking repeatedly at computer/notes and not the patient.
- Facial orientation: facing the patient, nodding, leaning forward to signal engagement.

### 3. Touch & Physical Contact

- Appropriate touch in clinical setting: handshake, pat on shoulder, holding patient's hand may signal empathy.
- But must be culturally sensitive and respect patient's boundaries.
- The references highlight that touch means different things in different cultures.

### 3. Touch & Physical Contact

- Clinical Application: Touch is powerful and complex.
- Appropriate: A hand on the shoulder during bad news, a handshake, and of course, the clinical physical exam. Always explain touch before performing it. ("I'm just going to feel your abdomen now.")
- Inappropriate: Touch that is patronizing, unwelcome, or unrelated to clinical care.
- Reading the Patient: Flinching from touch may indicate pain, past trauma, or cultural preference.

## 4. Proxemics

- Personal space is important. Standing too close may cause discomfort; too far may feel cold/uninterested.
- In clinical setting: sitting at eye-level, not behind a desk; ensuring patient has space.
- The reference says: “People often refer to their need for ‘personal space’ ... physical distance between you and others signals your level of intimacy and comfort.” Consider seating arrangement: e.g., clinician and patient sitting side by side can feel more collaborative.

# 4. Proxemics

- • Personal Zones:
  - • Intimate (0-1.5 ft): Physical exam.
  - • Personal (1.5-4 ft): The therapeutic zone for most history-taking.
  - • Social (4-12 ft): Speaking with a group or from the doorway.
  - • Public (12+ ft): Lectures.
- • Clinical Application: Never conduct the interview from the doorway. Pull up a chair and sit at or slightly below the patient's eye level. This simple act says, "I have time for you," and reduces the power differential.

# 5. Chronemics

- Clinical Application: Being on time for an appointment shows respect for the patient's time. Rushing through a consultation non-verbally screams, "You are not important." While time is limited, the perception of having enough time is created by giving the patient your undivided attention, even if just for a few minutes.
- How you handle time communicates messages: punctuality, waiting, pauses in conversation.
- For example: arriving late may be perceived as disrespect; pausing after asking a question gives patient time to reflect.
- From the reference: "Another type of non-verbal communication involves time... For example, begin late in work everybody a worker can be considered as a man of carelessness but if a manager does it; we say it is a symbol of power." In medicine: Allowing silence after delivering bad news gives space for the patient's emotional reaction.

## 6. Paralanguage

- This is about everything except the words themselves.
- Tone of Voice: The same sentence ("I'm sorry") can sound empathetic, sarcastic, or robotic.
- Pace: Speaking too fast seems hurried; too slow can seem condescending. Match the patient's pace when possible.
- Volume: Speak clearly, but avoid shouting.
- Pauses: Use strategic silence. It allows the patient to process information and gather their thoughts. Don't feel the need to fill every silence.



# 7. Physical Appearance & Environment

- Clinical Application: A professional appearance (clean white coat, neat attire) immediately conveys competence and respect. Your stethoscope is a non-verbal symbol of your role.
- The Environment: A clean, organized clinic room non-verbally communicates order and care. A cluttered, dirty room does the opposite.

## 8. Written/Visual Nonverbal

- Nonverbal communication is not only face-to-face: diagrams, charts, waiting room layout, clinic signage all send messages.
- For example: A very cluttered waiting room may convey disorganization; clear signage conveys professionalism.
- In teaching patients: Use visuals that are clear, non-threatening, culturally appropriate.

# 9. Silence

- Silence can speak volumes: allows processing, conveys empathy, respect.
- Used appropriately in patient consultations: after giving news, asking open-ended questions.
- Resist the urge to fill every pause with words; allow patient to reflect.

# Pitfalls

- Looking at screen/computer instead of the patient → patient may feel ignored.
- Crossed arms, leaning away → may convey disengagement.
- Monotone, rushed speech → patient may feel unheard or unsafe.
- Ignoring nonverbal cues from patient (e.g., discomfort, pain grimace).
- Inconsistent messages: “I am listening” but body turned away or clock watching.

# Strategies to improve

- Maintain open posture, face patient, lean slightly forward.
- Make appropriate eye contact (adjusting per culture).
- Moderate your tone, pace, volume; pause and allow reflection.
- Use simple but effective touch (if culturally appropriate).
- Keep your workspace neat, seat at appropriate level, sit rather than stand when possible.
- Observe the patient's nonverbal signals: adjust accordingly.
- Practice mindfulness: be aware of your own nonverbal behaviors.

THANK YOU